16.9 Cleft Lip and Palate*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Mother says, "The baby cannot suck well." **Objective** • Cleft lip and palate • Milk comes back through the nose	**ACTUAL** Ineffective infant feeding dynamics related to an anatomical defect as evidenced by a poor suck and nasal regurgitation.	• Enough intake, with weight gain.	• Use a special feeding bottle and teat. • Feed the baby upright. • Burp often. • Weigh regularly.	• Met when the baby gains weight.
Subjective • Mother asks, "Why did this happen to my baby?" **Objective** • Parents distressed • Mother reluctant to hold the baby	**ACTUAL** Excessive anxiety [parental] related to the infant's congenital defect as evidenced by distress and questioning.	• Parents voice their feelings and can describe the plan for repair.	• Explain when surgery is usually done. • Show photographs of repaired clefts if available. • Encourage holding and bonding.	• Achieved when the parents understand the plan.
Subjective *No specific complaint* **Objective** • Cleft palate	**RISK** Risk for aspiration as evidenced by risk factors: cleft palate.	• No aspiration.	• Feed upright. • Give small amounts and burp often. • Watch for coughing or color change.	• Outcome reached if aspiration does not occur.

* **References for this care plan:** [17], [35], [38]. Full entries are listed in the References section of the CPMN book.